

Patient ID: EG

Period: March 15 to May 6, 2024

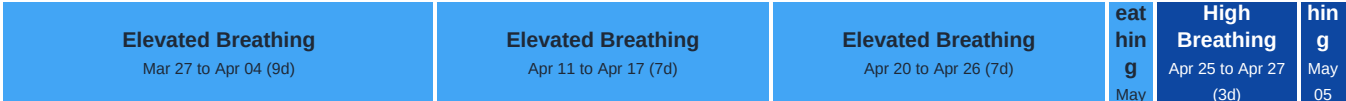
Report Date: May 7, 2024

Coverage: Bed: 915/1245h (73.5%)
Chair: 670/1245h (53.8%)

Patient's monitoring period was less than 90 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Legend: HR Breathing

Patient clinical status over 53 days from Mar 15 to May 06



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: GREEN

Vital	Avg	Min	Max
Heart Rate	60 bpm	48 bpm	79 bpm
Breathing	18 brpm	11 brpm	46 brpm

Last 24 hours: one very high breathing episode in the afternoon; one elevated breathing episode in the afternoon; one high breathing episode in the evening.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 05	1 PM to 2 PM	3h	3	Elevated Breathing	26	17	46	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 05	7 PM to 8 PM	1h	1	High Breathing (brief)	31	30	32	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 05	4 PM to 5 PM	1h	1	Very High Breathing (brief)	42	42	42	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

39 episodic events Detected, spanning 45h total. Conditions: Elevated Breathing, High Breathing, Low Heart Rate, Very High Breathing. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
39	45h	Elevated Breathing	<1	100%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 25	4 PM to 5 PM	2h	2	High Breathing	35	23	46	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 05	7 PM to 8 PM	1h	1	High Breathing	31	30	32	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Mar 27	12 PM to 2 PM	14h	10	Elevated Breathing	27	19	40	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 11	2 PM to 3 PM	14h	12	Elevated Breathing	26	15	44	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 20	6 PM to 7 PM	7h	7	Elevated Breathing	27	16	44	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 05	1 PM to 2 PM	2h	2	Elevated Breathing	26	17	46	Check SpO2 and lung sounds; assess for fluid overload or early infection

RR values outside physiologic range (6–60 breaths/min).

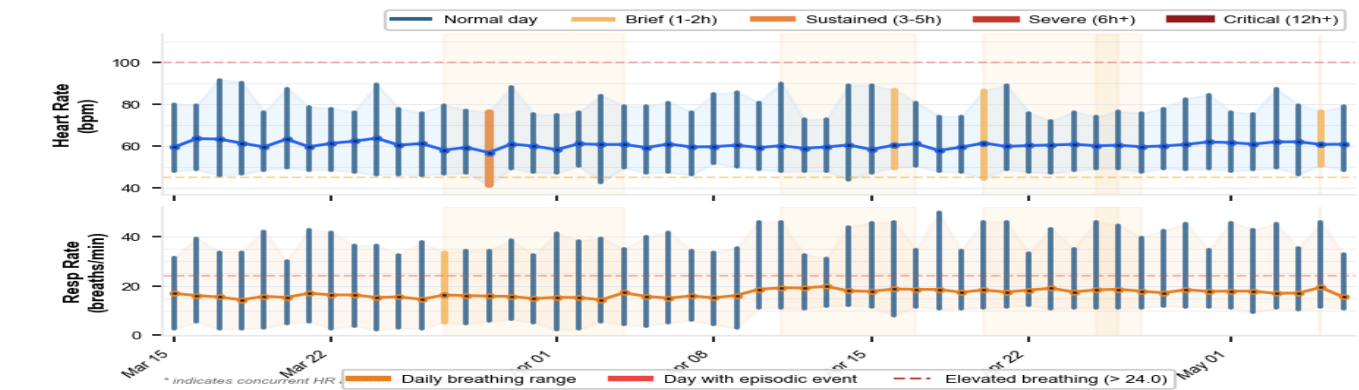
Suggested Clinical Review Actions

- Elevated Breathing (avg 27 brpm, peak 40 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Low Heart Rate (avg 44 bpm, min 41 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Very High Breathing (avg 44 brpm, peak 46 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

- High Breathing (avg 35 bpm, peak 46 bpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 6 unstable phases with 17 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

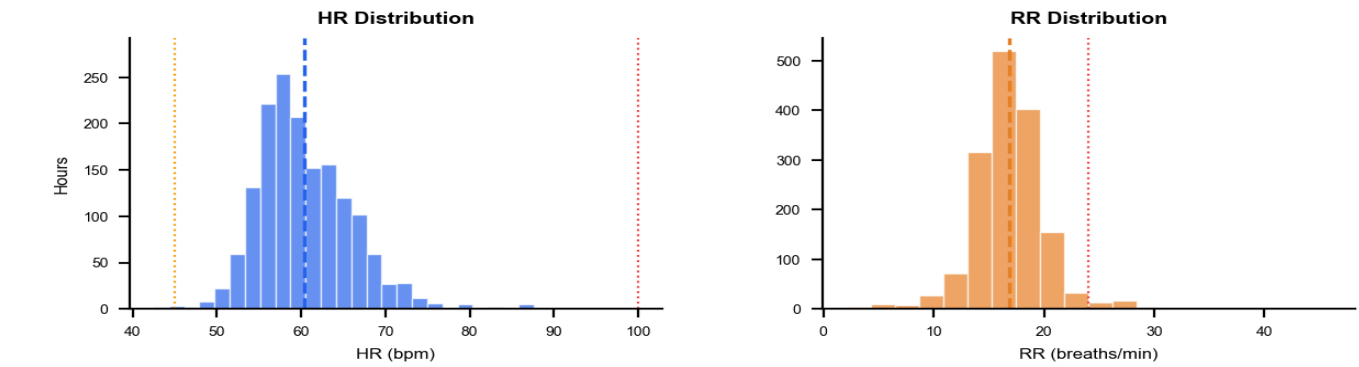
Clinical Pattern Observations:

- 1. **Clustered pattern:** Episodic events on 5 of 53 days (9%).
- 2. **Daytime pattern:** Episodes concentrated during waking hours.

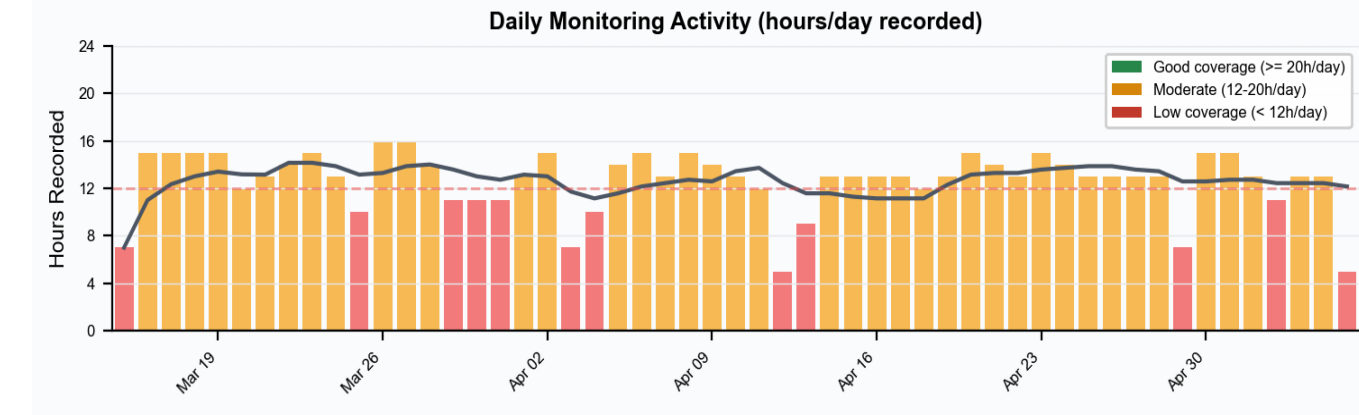


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

